

3.2 Financial Model and Commercial Viability

3.2.1 Revenue Model and Pricing

The proposed revenue model is a B2B2C institutional subscription model. The end users are isotretinoin patients and dermatology clinicians and the paying customers are the healthcare providers, that is, NHS dermatology department, private dermatology clinic and online dermatology. It is more suitable than the direct to consumer because, under the structure of our app, the app supports a regulated treatment-monitoring pathway and not a general acne self management app.

We assume in the model one core product: an annual provider subscription for the isotretinoin monitoring platform. The base-case price is £24,000 per provider per year, or £2,000 per month. This subscription structure has been chosen because it is easier for the providers to budget for, and because the company does not have to charge the patients. It also means that we do not need to know at every provider, in the early stage of the price, how many of the isotretinoin patients they have.

To sense-check this price, the model assumes that each London dermatology provider may manage approximately 150-300 isotretinoin-relevant patients per year. This is an assumption that is scenario based. It is derived from the (estimated) London serviceable available market (SAM) of around 6,000-9,000 (England $\sim 75,000$ extra appointments/yr $\div \sim 1.5-2$ (estimate) per patient $\approx \sim 38,000-50,000$ /yr $\times$ London 15-18% $\approx \sim 6,000-9,000$ /yr) isotretinoin-relevant patients distributed across the NHS dermatology departments, private dermatology clinics and online dermatology providers (Schofield and Bunker, 2025). Hence, a £24,000 per year fee will equal approximately £80-£160 per patient per year. Assuming an active monitoring period of about six months, this will in effect be approximately £13-£27 per active patient per month.

This implied monthly cost was compared with remote patient monitoring (RPM) software-as-a-service (SaaS) pricing benchmarks. MedM Care lists RPM pricing at around 10 per patient per month, and HealthArc's RPM pricing guide suggests software platform fees of about 10-40 per patient per month (MedM Care, n.d.; HealthArc, 2025). These products are not isotretinoin-specific, but they are relevant benchmarks

for digital remote monitoring software. The implied 13-27 per active patient per month therefore seems reasonable as a base-case pricing assumption.

There are also pricing sensitivity scenarios in the model. For example, a low pilot price of £12,000 per year could be offered to smaller clinics or early pilot partners, and £36,000 per year could be used for larger providers or multi-site organisations once clinical and operational value has been shown. The base case of £24,000 is therefore, as an assumption, placed between the affordability of the early customers and the commercial sustainability of the business.

Table 1. Pricing scenarios (£ per provider per year).

Scenario	Annual fee	Suitable customer	Use
Low / pilot	£12,000	Small private clinic / first pilot	Supports early adoption, but may underestimate long-term value
Base	£24,000	Standard clinic / online dermatology provider / NHS dermatology service	Used in the financial model
High	£36,000	Large NHS Trust / multi-site provider	Upside scenario after validation

Table 2. Pricing benchmarks. Source: MedM Care (n.d.), HealthArc (2025), Teladoc Health (2025), Hims & Hers Health (2026), Doximity (2025), and team calculations.

Comparable	Pricing basis	Figure	Relevance
MedM Care	RPM, per patient per month	approx. \$10	Patient-level RPM pricing benchmark
HealthArc	RPM software platform licence, per patient per month	approx. \$10-\$40	Broad RPM SaaS pricing range
Our app	Implied per patient per month, six-month course	approx. £13-£27	Falls within a plausible RPM benchmark range
Teladoc Health	Access fee / platform revenue	2025 access-fee revenue \$2.09bn	Supports institutional access-fee logic
Hims & Hers	Digital health subscription platform	Q4 2025 gross margin 72%	Supports scalable digital health margin potential
Doximity	Healthcare SaaS subscription model	FY2025 revenue \$570.4m; subscription revenue \$543.8m	Supports institutional healthcare SaaS scalability

Note: RPM comparators are not identical to the proposed isotretinoin app. They are used as external price benchmarks for remote monitoring software. Dollar values are not directly converted in the table; the app's implied £13-£27 per active patient per month should be interpreted as an approximate sanity check rather than an exact like-for-like comparison.

3.2.2 Cost Structure

The cost structure is split in variable costs and fixed operating costs. The variable costs are captured under cost of good sold (COGS). In the base-case model we estimate COGS to be of £5,260 per provider. These are the costs of cloud hosting, onboarding, patient support, reminder systems, basic technical support and other costs of the delivery of the usage. If the price is taken as annual of £24,000 and COGS of £5,260 per provider the model produces a gross margin of 78.1%. This is in line with the fact that the product is a scalable software based service instead of a labour intensive clinical service.

Fixed operating expenses are split into: General and administrative costs, staff costs, marketing costs and other central costs. General and administrative costs are: accounting, legal, insurance and administrative software. Staff costs are the core team required to develop, maintain and commercialise the platform. The model assumes that we are hiring in phases rather than a large team from the start. We do this to help manage the early stage cash burn. Marketing costs are provider outreach, branding, events, conference attendance and sales materials. Other central costs are for the other shared overheads such as internal systems, professional services and contingency.

The model also recognises the importance of upfront development and compliance costs. Because the app handles sensitive health data and supports a clinical monitoring pathway costs related to data protection, cybersecurity, clinical safety review, GDPR compliance, penetration testing, professional liability insurance and legal advice are included within the broader operating cost assumptions. It is important because health data is treated as special category data under UK data protection rules, and high risk processing is more likely to require a data protection impact assessment (ICO, 2023). If the product is implemented in NHS settings, clinical safety documentation may also be required under NHS digital clinical safety standards such as DCB0129 (NHS England, 2025).

3.2.3 Three-Year Financial Projection

The three-year forecast is based on gradual provider adoption. The model assumes 3 paying providers in FY1, 8 in FY2 and 15 in FY3. The revenue therefore increases from £72,000 in FY1 to £192,000 in FY2 and £360,000 in FY3. The total revenue across the three-year forecast period is £624,000.

Even though revenue grows, the business is still loss-making across the first three years. The earnings before interest, tax, depreciation and amortisation (EBITDA) is negative in each year, but the loss narrows from £165,972 in FY1 to £149,259 in FY2 and £139,675 in FY3. This shows that the business does move in the right direction as the adoption increases, even though it has not yet reached break-even within the three years. This is expected for an early stage digital health venture because product development, compliance, staffing and provider acquisition costs take place before the platform reaches scale.

The model assumes and £300,000 of equity funding and £75,000 of grant funding. These funding sources support the business during the early loss making phase and allow the company to continue product development, clinical validation and market entry. Closing cash is positive at approximately £220,094 by the end of FY3 so the business does not use up its cash runway during the forecast period. The financial model therefore supports the long term development strategy rather than the short term profitability.

Table 3. Three-year financial projection, base case (£).

	FY1	FY2	FY3	Total
Revenue	72,000	192,000	360,000	624,000
Less: COGS	(15,780)	(42,080)	(78,900)	(136,760)
Gross profit	56,220	149,920	281,100	487,240
G&A	(48,696)	(48,024)	(77,796)	(174,516)
Staff	(84,096)	(135,955)	(156,979)	(377,030)
Marketing	(28,200)	(57,600)	(108,000)	(193,800)
Other central	(61,200)	(57,600)	(78,000)	(196,800)
Total OpEx	(222,192)	(299,179)	(420,775)	(942,146)
EBITDA	(165,972)	(149,259)	(139,675)	(454,906)
Grants	50,000	25,000	0	75,000
Net profit after tax	(115,972)	(124,259)	(139,675)	(379,906)
Closing cash	334,028	309,769	220,094	-

Note: Brackets indicate negative values. Total OpEx is calculated as G&A + Staff + Marketing + Other central costs. The EBITDA values therefore use the corrected operating expense logic rather than the original template formula.

3.2.4 Adoption Assumptions

The adoption assumptions are deliberately conservative. The model assumes 3 paying providers in FY1, 8 in FY2 and 15 in FY3. Against an estimated addressable pool of 40–50 London dermatology providers — derived from around 80 NHS dermatology units in England (GIRFT, 2021) scaled to London, plus private clinics and online platforms — the FY3 target of 15 providers represents a conservative minority share. The adoption pathway reflects a realistic B2B digital health rollout: first validate the product with a small number of (pilot) providers, then use the early evidence and case studies to support the rest. The detailed provider acquisition strategy is discussed in the implementation section.

3.2.5 Key Financial Assumptions

There are five assumptions in the financial model. First, the average selling price is assumed to be at the level of £24,000 per provider per year throughout the forecast period. This is conservative because the model does not assume that the price will increase. Second, the provider adoption will increase from 3 in FY1 to 8 in FY2 and 15 in FY3. Third, the gross margin is at 78.1%, this is based on COGS of £5,260 per provider. This gross margin is broadly consistent with the scalable digital health business models, for example, Hims & Hers report the gross margin of 72% in Q4 2025, and Doximity report the revenue of FY2025 is \$570.4 million, and it is largely from the subscription revenue (Hims & Hers Health, 2026; Doximity, 2025). Fourth, the staff cost will increase gradually as the company is moving from develop to pilot delivery and towards commercial expansion. Fifth, the company will get the equity funding of £300,000 and the grant funding of £75,000 for the early stage losing and product development.

3.2.6 Financial Risks and Mitigation

The first major financial risk is slow provider adoption. Procurement cycles in healthcare can be very long, particularly in the NHS setting. The business should therefore take private dermatology clinics and online dermatology providers for the early pilots, and use a lower pilot price of £12,000 to lower the bar to adoption.

A second risk is that compliance and cybersecurity costs may be higher than expected. The platform deals with sensitive health information, therefore data protection, clinical

safety, cybersecurity and legal requirements could increase costs. This can be addressed by phased product development, early compliance advice and a dedicated contingency budget.

A third risk is that the business may remain loss making beyond the three year forecast period if provider acquisition is slower than expected. This can be dealt with by staged hiring, grant applications, bridge funding and careful control of operating expenses.

Overall, the model shows that the business is not immediately profitable, but that it has a plausible path toward commercial viability if adoption increases and cost growth is controlled. A third risk is that the business may remain loss making beyond the three year forecast period if provider acquisition is slower than expected. This can be dealt with by staged hiring, grant applications, bridge funding and careful control of operating expenses.

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